



Diagnostic Audit — Approach & Framework

For review: Dr Debo Odulana & Tito Ipinmoye · evidence-based, MECE, hypothesis-driven

The read, in one line: this is a strategic-alignment diagnosis, not an operational checklist. Haven's visible problems are alignment gaps between where it wants to go and how it is built to run. Every finding lands as a fit gap, an integration gap, or both. Culture is the multiplier.

1. The MECE structure — two axes

Axis A — ten functional domains (mutually exclusive by function, following the value chain): 1 Demand & Market · 2 Access & Patient Flow · 3 Clinical Care & Governance · 4 Pharmacy & Supply Chain · 5 Revenue Cycle · 6 Finance & Capital · 7 People & Organisation · 8 Infrastructure, Equipment & Utilities · 9 Digital & Technology · 10 Governance & Compliance.

Axis B — six cross-cutting lenses (examined through every domain): culture & incentives · cash / working capital · data integrity · risk & safety · stakeholder experience · scalability / growth-readiness.

The audit is the **matrix**: each domain read through each lens. Every cell is unique (mutually exclusive); the grid is exhaustive. This is what makes "culture is the multiplier" structural — culture is a lens run through all ten domains, not one competing box.

2. Method & signature KPI per domain (evidence-based)

Domain	Method / standard we apply	Signature KPI
1 Demand & Market	Catchment, referral-source & payer-mix analysis	Referral volume; win-rate; payer mix
2 Access & Patient Flow	Lean value-stream map + takt-time; stress-test at 2x demand	Waiting time; lead time; takt vs cycle
3 Clinical Care & Governance	Protocol / incident / M&M; audit vs COHSASA / SafeCare	Adverse events; protocol adherence; outcomes
4 Pharmacy & Supply Chain	Stock spot-count vs system; VMI case; margin decomposition	Stockout rate; days-of-stock; true margin
5 Revenue Cycle	Claim-to-cash process map; denial analytics	Clean-claim %; denial %; days-in-AR
6 Finance & Capital	Cash-conversion & working-capital analysis; unit economics	Cash-conversion cycle; WC locked; contribution / line
7 People & Organisation	McKinsey 7S diagnostic; org & consultant-model review	Staffing ratios; turnover; 7S soft-element gaps
8 Infrastructure, Equipment & Utilities	WHO oxygen fit-for-use audit; layered-power resilience; biomedical maintenance	O2 purity ($\geq 82\%$); outage freq/duration; % kit operational
9 Digital & Technology	Systems / data audit; reporting reconciliation	Reporting integrity; data availability
10 Governance & Compliance	Board / ownership & regulatory review (HEFAMAA, NDPR)	Governance maturity; compliance status

Research flag — Infrastructure (Domain 8) is a top-tier CLINICAL risk, not a facilities footnote. In Nigerian secondary hospitals only ~3.5% of oxygen concentrators meet WHO fit-for-use; power outages interrupt oxygen with ~19% case-fatality among affected patients; paediatric hypoxemia raises death risk ~5x. We audit medical oxygen ($\geq 82\%$ purity, WHO-UNICEF 2021) and layered power resilience (grid + generator + battery — solar alone is not turnkey) as first-order clinical safety, and equipment for operability (only 10-30% of donated SSA kit stays operational).

3. How we run it

Process work uses **Lean value-stream mapping + takt-time** — validated both to redesign a pathway and to **stress-test it for scale** (higher demand shortens takt time, pushing more stages over the bottleneck threshold, so



we can see what breaks at 2x volume before it does). One caution the evidence is clear on: Lean fails in hospitals when it is imposed as a manufacturing tool — staff read it as a threat to clinical autonomy — so we run it collaboratively (the culture lens applied to the method itself).

Operating-model & growth-readiness use the McKinsey 7S framework (an empirically validated hospital diagnostic that surfaces soft-element gaps financials miss) and a **staged maturity model** (3-9 levels) to place Haven today and chart the path to a scalable state.

4. The hypothesis bank — what we are trying to prove or kill

Every interview and data pull tests one of these. Full detail in the interview guides.

- **Clinic, not a business:** reporting doesn't reconcile, so leadership flies blind.
- **Cash conversion, not profit,** is the killer — ~N7M trapped; growth consumes WC before it returns it.
- **Capacity ahead of demand:** NICU sold as the engine without a proven referral pipeline.
- **Visiting-consultant paradox:** the model that built Haven caps it — safety can't be standardised on sessional consultants.
- **Incentives reward the wrong things** — activity over quality, safety and ownership; growth scales bad behaviour.
- **Process fragility:** core processes work by heroics; they break, not bend, at scale.
- **No payer power** in a 94-HMO market — Haven is financing the HMOs.
- **No one's full-time job is to run Haven** — owners firefight; systems never get built.
- **Safety is the licence to grow** — until it is a system, every admission adds tail risk.
- **Infrastructure is the silent existential risk** — oxygen + power resilience for NICU (research-validated).
- **Haven hasn't chosen what it wins at** — premium, volume, or referral NICU need different models.

5. Stakeholder instruments (the experience lens)

Five validated instruments so results benchmark, not just collect: **patients/caregivers** (Child-HCAHPS / Picker PPE-15) and **staff** (AHRQ HSOPS 2.0) — **both live now**; plus **visiting consultants, payors/HMOs** (provider-payer, both directions + claims analytics) and **referring clinicians** (validated referrer surveys + NPS) — to build.

6. Honest evidence gaps (and how we close them)

Two gaps we are transparent about. (1) **No reliable Nigerian revenue-cycle benchmarks** exist publicly (clean-claim, denial, days-in-AR) — so we set Haven's baseline from its own data and track improvement, not against an imported number. (2) **"MECE" is our analytic architecture** — the functional domains are well-supported by evidence, but the two-axis matrix is a consulting framing, not an externally certified standard; its rigour is in the domain/lens definitions we have locked here.

Companion documents: the **Interview Guides** (attached), the client-facing **Kickoff Deck** and **Information & Data Request**, and the two live surveys. Please mark up anything here — especially the hypotheses; they are meant to be argued with before we lock them.